SARMs SCIENTIFIC HANDBOOK

Ch 1: What Are SARMs?

Selective Androgen Receptor Modulators (SARMs) bind to androgen receptors with tissue selectivity — targeting muscle and bone while theoretically sparing other androgen-sensitive tissues. Originally developed for muscle-wasting diseases and osteoporosis.

Comparison	SARMs	Anabolic Steroids
Oral bioavailability	High for most SARMs	Varies; injectables require different delivery
Liver toxicity	Low to moderate	Moderate to high for oral 17-alpha alkylated compounds
Aromatisation	None for most SARMs	Yes for testosterone-based compounds
HPTA suppression	Moderate (dose/compound dependent)	Severe — shuts down natural production
Androgenic side effects	Low	High — hair loss, acne, prostate effects
Anabolic potential	Moderate	High
Legal/regulatory status	Research chemical; not FDA approved for humans	Controlled substances (Schedule III in USA)

Ch 2: SARMs Profiles

SARM	Dose mg/day	Half-life	Best Application	HPTA Suppression	Key Note
Ostarine (MK-2866)	15–25	24 hrs	Recomp; beginner first SARM	Low–Moderate	Mildest; joint healing benefit
RAD-140 (Testolone)	10–20	60 hrs	Strength and mass gain	High	Most anabolic; monitor liver
LGD-4033 (Ligandrol)	5–10	24–36 hrs	Lean mass gain	Moderate–High	Dose-dependent suppression
Cardarine (GW-501516)	10–20	16–24 hrs	Endurance; fat loss	None (not a SARM)	PPARdelta agonist; cancer studies controversy
MK-677 (Ibutamoren)	15–25	24 hrs	GH; sleep; recovery; mass	None (not a SARM)	GH secretagogue; no suppression
S-23	10–30	12 hrs	Cutting; muscle hardness	Severe	Most suppressive SARM; full PCT
YK-11	5–15	6–10 hrs	Extreme mass gain	High	Partial myostatin inhibitor
Andarine (S-4)	25–50	4 hrs	Cutting and recomp	Moderate	Yellow vision tint side effect
SR-9009 (Stenabolic)	20–30 (split 4x)	4–6 hrs	Fat loss; endurance	None	Low oral bioavailability issue

Ch 3: Stacking Protocols

Goal	Stack	Duration	PCT Needed
Beginner recomp	Ostarine 15mg + Cardarine 10mg	8 weeks	Mini-PCT (Nolvadex 20mg x 4 weeks)
Lean bulk	LGD-4033 10mg + MK-677 20mg	12 weeks	Full PCT (Nolvadex 40/40/20/20)
Aggressive bulk	RAD-140 15mg + LGD-4033 5mg + MK-677 20mg	10 weeks	Full PCT required
Cutting phase	Ostarine 20mg + Cardarine 20mg + SR-9009 20mg	8 weeks	Mini-PCT
Recomposition	RAD-140 10mg + Cardarine 15mg	8 weeks	Full PCT
Endurance only	Cardarine 20mg + SR-9009 30mg	6–8 weeks	No PCT needed

Ch 4: Health Monitoring

Required Blood Tests On SARMs
• Pre-cycle: Total T, Free T, LH, FSH, E2, SHBG, liver enzymes (ALT/AST), CBC, lipids
• Mid-cycle (week 4–6): Total T, liver enzymes, lipids — check for suppression or liver stress
• Post-PCT (4 weeks after): Total T, LH, FSH, E2 — confirm HPTA recovery
• Concerning: Total T < 300 ng/dL post-PCT; ALT/AST > 3x baseline; HDL drop > 30%
• Recovery timeline: LH/FSH usually return within 4–8 weeks post-SARM cycle

PCT Type	When to Use	Protocol	Duration
Mini-PCT	Mild stacks (Ostarine-only, low dose)	Nolvadex 20mg/day	4 weeks
Full PCT	Moderate-high suppression (RAD, LGD, S-23)	Nolvadex 40/40/20/20 + Clomid 100/50/50/50	4 weeks
Natural recovery only	Non-suppressive compounds (Cardarine, MK-677, SR-9009)	No PCT; supplement support stack only	4–8 weeks

Ch 5: SARMs Safety Considerations

All SARMs currently available are sold as research chemicals — not approved for human use by any regulatory agency. This means product quality is highly variable between suppliers, and long-term human safety data is largely absent. This section covers harm-reduction practices.

Risk	Evidence Level	Harm Reduction
Liver toxicity (mild-moderate)	Some studies show ALT elevation	Run liver support: TUDCA 250mg + NAC 400mg daily on cycle
HPTA suppression	Well-established	Always run PCT; blood test before and after
Cardiovascular (lipid changes)	HDL reduction documented	Omega-3 4g/day; cardio; monitor blood lipids mid-cycle
Long-term safety unknown	No long-term human studies	Limit cycle lengths; time off between cycles
Counterfeit or mislabelled products	Very common in grey market	Purchase only from vendors with HPLC testing certificates

Signs of SARM Suppression (Get Blood Work If You Experience These)

- Significant reduction in libido or sexual function during cycle
- Fatigue, lethargy, and low motivation not explained by training load
- Testicular ache or noticeable size reduction
- Mood changes: irritability, depression, emotional blunting
- These are hormonal indicators — do not wait; get LH, FSH, Total T tested